Trastuzumab deruxtecan (Enhertu) · every 3 weeks

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Trastuzumab deruxtecan (Enhertu) · every 3 weeks

Active medicines: Trastuzumab deruxtecan (Enhertu)

Emetogenic risk: High · dedicated trastuzumab deruxtecan row

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 before treatment
Ondansetron	16 mg orally	Day 1 before treatment Review QT risk, hepatic impairment, constipation and interacting medicines.
Dexamethasone	12 mg orally	Day 1 before treatment Reconcile all steroid premedication; do not duplicate.
Aprepitant	80 mg orally once daily	Days 2–3
Dexamethasone	8 mg orally once daily	Days 2–4 Confirm suitability and total steroid exposure.
Metoclopramide	10 mg orally up to three times daily	Rescue use / duration to be completed by prescriber Review renal function and neurological contraindications; avoid automatic prolonged use.

Additional phase-specific support

- This regimen has high emetic risk. An antiemetic combination is prescribed before every infusion, with an agreed plan for delayed symptoms and breakthrough nausea. The dedicated National Cancer Control Programme (NCCP) trastuzumab-deruxtecan pathway should be checked by the prescriber and pharmacist.
- Carry the patient alert card and report new cough, fever or breathlessness promptly. Steroids used for suspected lung inflammation are prescribed and monitored by the clinical team; do not start them yourself from this information.
- Low blood counts can cause infection, anaemia or bleeding. The need for granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or transfusion support depends on results, symptoms and previous complications; it is not automatic for every cycle.
- Small frequent meals and early treatment of nausea can help maintain intake. Tell the team about weight loss, constipation or diarrhoea so that support can be tailored.
- NCCP NCIS v8 Table1 has a dedicated trastuzumab-deruxtecan row: NK1 + 5-HT3 + dexamethasone. Olanzapine can be considered by the prescriber, but is not silently added to that national default row.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00776 v3b · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required